

SOP and Handover Toolkit

Operational procedure, checklist, RACI, escalation, training, and audit pack

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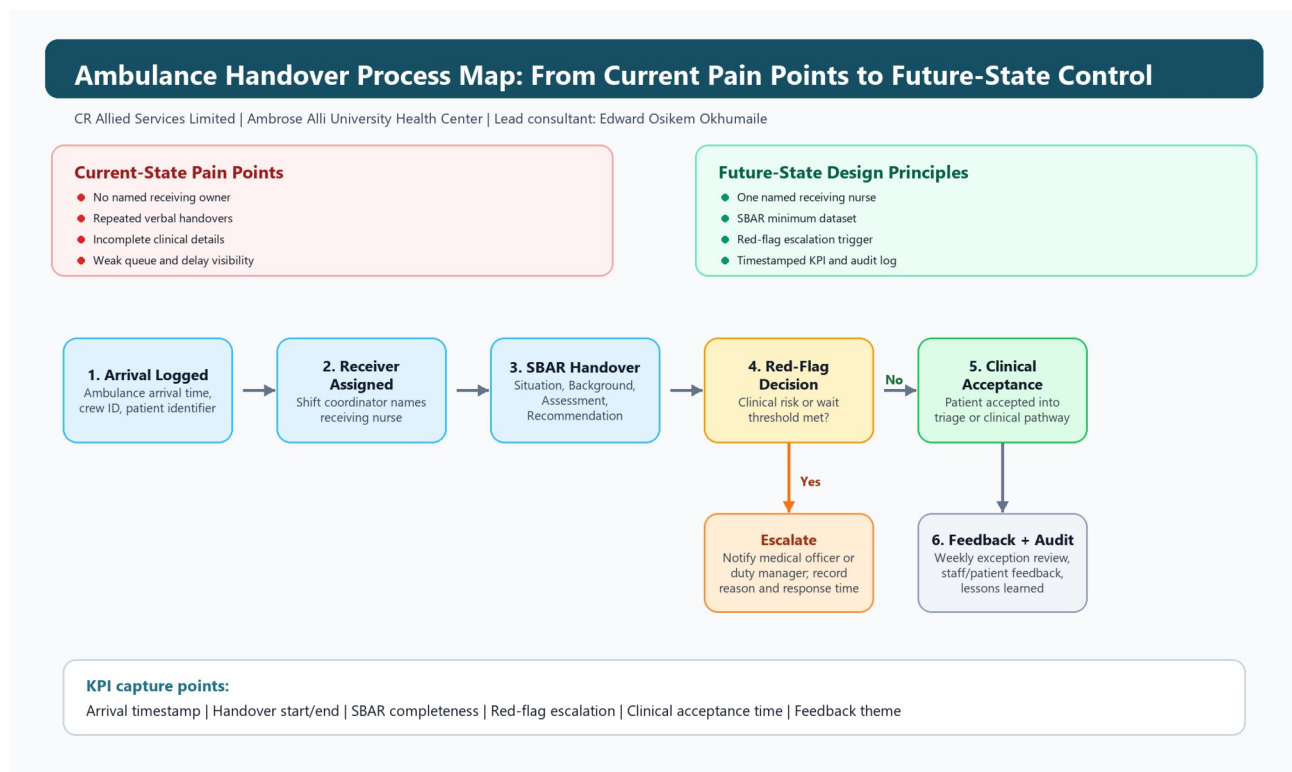
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Clinical governance note: This SOP template supports operational handover coordination. It does not replace local clinical judgement, emergency clinical protocols, or regulatory requirements.

1. SOP Purpose

To ensure each ambulance arrival is received, clinically acknowledged, documented, escalated where required, and transferred into the health centre pathway using a consistent, auditable process.

Figure 1. SOP process map for ambulance handover.



Staff should use this map as a quick visual guide to understand where ownership, SBAR documentation, escalation, clinical acceptance, and audit reporting occur.

2. Standard Procedure

1. Security or receiving desk logs ambulance arrival time and alerts the shift coordinator.
2. Shift coordinator assigns a receiving nurse and records the named receiver.
3. Ambulance crew gives SBAR handover to the receiving nurse using the standard template.
4. Receiving nurse records core data, red flags, vital signs, allergies, medication, and immediate risks.
5. If escalation threshold is met, receiving nurse alerts the medical officer or senior clinician immediately.
6. Triage completes clinical acceptance and confirms patient pathway.
7. Quality lead reviews weekly exceptions, missing records, delays, and feedback themes.

3. SBAR Handover Checklist

Table 1. Minimum handover dataset.

SBAR area	Required information	Completed
Situation	Patient identifier, age band, presenting complaint, arrival mode, infection risk	Yes / No
Background	Known conditions, medication, allergies, recent events, referral source	Yes / No
Assessment	Vital signs, pain score, consciousness level, mobility, immediate concerns	Yes / No
Recommendation	Requested action, urgency, red flags, clinician escalation required	Yes / No
Operational	Arrival time, handover start/end, receiving nurse, crew ID, queue status	Yes / No

4. RACI

Table 2. Role accountability matrix.

Activity	Ambulance crew	Receiving nurse	Shift coordinator	Medical officer	Quality lead
Provide SBAR handover	R	A	C	I	I
Assign receiving nurse	I	C	A/R	I	I
Document minimum dataset	C	A/R	I	I	I
Escalate red flags	C	A/R	I	R	I
Review weekly KPI report	I	C	C	C	A/R
Update SOP after lessons learned	I	C	C	C	A/R

5. Escalation Thresholds

Table 3. Escalation triggers.

Trigger	Action	Expected response
Airway, breathing, circulation concern or reduced consciousness	Immediate clinician alert	Clinician attends without delay
Handover wait exceeds 15 minutes	Notify shift coordinator	Queue pressure reviewed and receiver assigned
Handover wait exceeds 30 minutes	Escalate to duty manager / senior clinician	Capacity action agreed
Incomplete critical information	Request clarification from crew before clinical acceptance	Missing critical fields resolved or exception recorded
Patient/carers distress or complaint	Inform nurse in charge and record feedback	Support provided and feedback logged

6. Training Plan

Table 4. Role-based training plan.

Audience	Training content	Duration	Evidence
Receiving nurses	SBAR form, red flags, escalation, documentation quality	45 minutes	Attendance sheet and competency sign-off
Ambulance crew liaison	Handover expectations and minimum dataset	30 minutes	Briefing record
Shift coordinators	Queue management, escalation, KPI review	45 minutes	Scenario practice
Managers / quality team	Dashboard interpretation and benefits review	30 minutes	Dashboard review minutes

7. Audit Form

Table 5. Weekly audit sample form.

Audit item	Target	Result	Action owner
Arrival logged within 1 minute	>= 90%		
SBAR core fields complete	>= 90%		
Median handover time	<= 15 minutes		
Red-flag escalations documented	>= 95%		
Feedback themes reviewed	Monthly		